COPD — GOLD Grade, ABE Group & SA STG Treatment

adults with spirometry-confirmed COPD · OPD / casualty

GOLD 1–4 · A/B/E

DIAGNOSIS

Airflow obstruction ✚: **post-bronchodilator $FEV_1/FVC < 0.70$**

SPIROMETRIC GRADE (FEV_1 % PREDICTED)

GOLD	Severity	FEV_1
1	Mild	≥80%
2	Moderate	50–79%
3	Severe	30–49%
4	Very severe	<30%

MMRC DYSPNOEA SCALE ✚

Grade	Breathless…
0	only with strenuous exercise
1	when hurrying on level ground or walking up a slight hill
2	walks slower than peers, or stops for breath at own pace
3	stops after ~91 m (100 yards) or a few minutes on level ground
4	too breathless to leave the house, or when dressing

GROUP (EXACERBATIONS IN PAST YEAR) ✚

	mMRC 0–1	mMRC ≥2
<2, no hospital admission	**A**	**B**
≥2, or ≥1 leading to hospital admission	**E**	**E**

SA STG — MAINTENANCE

- All: salbutamol MDI 200 mcg 6-hourly as needed ✚
- Group B: + formoterol 12 mcg 12-hourly ✚
- Group E: eosinophils <0.1 → formoterol; ≥0.1 × 10^9/L → salmeterol/fluticasone 50/250 12-hourly ✚

SA STG — EXACERBATION

- Controlled O_2 1–2 L/min nasal; target SpO_2 90% / PaO_2 8 kPa ✚
- Salbutamol 5 mg neb (refill every 20 min); poor response → add ipratropium 0.5 mg ✚
- Prednisone 40 mg daily 5 days (or hydrocortisone 100 mg IV 6-hourly) ✚
- Antibiotic if ↑purulence + ↑volume or dyspnoea, or severe: amoxicillin/clavulanate 875/125 mg 12-hourly 5 days (allergy: azithromycin 500 mg daily 3 days) ✚
- STG table header misprinted ('Grade Exacerbations in past year') ‡ ✚

HOSP STG & EML Hospital Level Adults, 6th ed. 2024 (updated Mar 2026) — pp 16.10, 16.11, 16.12, 16.9 · **GOLD** GOLD 2025 Report, Global Initiative for Chronic Obstructive Lung Disease
✚ = SA STG / national guideline (pages listed above). Unmarked content: international source named above. ‡ = STG misprint or sources differ (see app). ANDH · copd-gold · updated 2026-10-14